A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

30 April 2026

Prof Matthew Read
Epworth Freemasons

Dear Prof Read,

Re: Mr Nathan Poratt
DOB: 07/04/1960 Phone: 0418 343 608
Address: 23 Royal Parade, Caulfield South, VIC, 3162
Email: natsue88@bigpond.com
Medicare: 3503677834 / 1

Summary

- **Recurrent GORD following revisional hiatus hernia repair with associated visceral hypersensitivity.**
- **Commenced on optimised proton pump inhibitor therapy and low-dose neuromodulation.**
- **No action required by you.**

I had the pleasure of conducting an in-person consultation with Nathan Poratt, thank you for referring him.

Symptoms and history of presenting complaint

Nathan has a decades-long history of gastro-oesophageal reflux disease (GORD). His surgical history is significant, having undergone an initial hiatus hernia repair with Mr Liang Low at Cabrini, which was complicated by severe post-operative dysphagia and significant weight loss. He subsequently underwent a successful revisional repair with you. While the revision successfully resolved his dysphagia and allowed him to return to a baseline weight of 85kg, his primary reflux symptoms have since recurred.

He describes a classic constellation of symptoms including acid regurgitation, retrosternal burning, and epigastric pain. These symptoms typically manifest approximately four hours after eating and are particularly troublesome in the early afternoon and overnight. He also reports persistent abdominal bloating that has been present since his initial surgery. Nathan notes that his symptoms escalated significantly eight to nine years ago, coinciding with a period of profound personal stress involving a divorce and family bereavement, suggesting a strong component of visceral hypersensitivity.

Diet

Nathan's dietary habits are characterised by a lack of breakfast and lunch, consuming only three cups of black coffee and some fruit during the day. He consumes his largest meal at dinner between 5:30 pm and 6:30 pm. He has noticed significant bloating regardless of intake, which often discourages him from eating later in the day.

Past medical history and medications

Nathan's medical history is notable for Chronic Lymphocytic Leukaemia (CLL), which is currently under stable surveillance by his haematologist. He also suffers from benign prostatic hyperplasia (BPH) and is under the

A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

care of a urologist. His current medications include pantoprazole 40mg (which he has been taking sub-optimally after food), Duodart, and tamsulosin. He has previously used esomeprazole and ranitidine with varying degrees of success. He has no known allergies.

Social history, family history

Nathan works as an electrical contractor and lives with a supportive partner. He is a non-smoker and infrequent consumer of alcohol. His family history is significant for maternal arc (deceased at age 62). His son also suffers from mild reflux. There is no family history of inflammatory bowel disease or coeliac disease.

Examination On examination, Nathan appeared well. His abdomen was soft and non-tender with no palpable masses or organomegaly. Bowel sounds were normal. There was no evidence of peripheral stigmata of chronic liver disease or malabsorption.

Impression and plan

Nathan's presentation is consistent with refractory GORD following revisional hiatal surgery. Given the temporal relationship of his symptoms to major life stressors, there is likely a significant element of visceral hypersensitivity, where the oesophageal mucosa has become hyper-responsive to even physiological levels of reflux. Furthermore, his current PPI therapy has been ineffective, likely due to poor timing of administration and the fact that pantoprazole is more susceptible to genetic metabolic variations that can reduce its efficacy.

Our plan is as follows:

- **Pharmacotherapy:** I have transitioned Nathan from pantoprazole to rabeprazole 20mg twice daily, with strict instructions to take the dose 30 minutes before breakfast and dinner to ensure maximal acid suppression. If his symptoms stabilise after one month, he may reduce this to a once-daily dose before dinner.
- **Neuromodulation:** To address the visceral hypersensitivity, I have commenced a low-dose trial of amitriptyline 10mg at night. We discussed potential side effects, including morning grogginess and the potential impact on his urinary stream given his history of BPH.
- **Lifestyle:** I have advised Nathan to redistribute his caloric intake to earlier in the day and to reduce the size of his evening meal to decrease the nocturnal reflux burden.
- **Follow-up:** We will review his progress in approximately eight weeks. At that time, we will also schedule a surveillance colonoscopy, which is indicated given his maternal family history of malignancy.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Sebastian Pavone

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065